

# PEER EDUCATOR TRAINING MANUAL

## FOR PREVENTION OF HIV AND AIDS

among heroin smokers and injecting drug users



CARE Bangladesh

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HIV Program  
**CARE Bangladesh**

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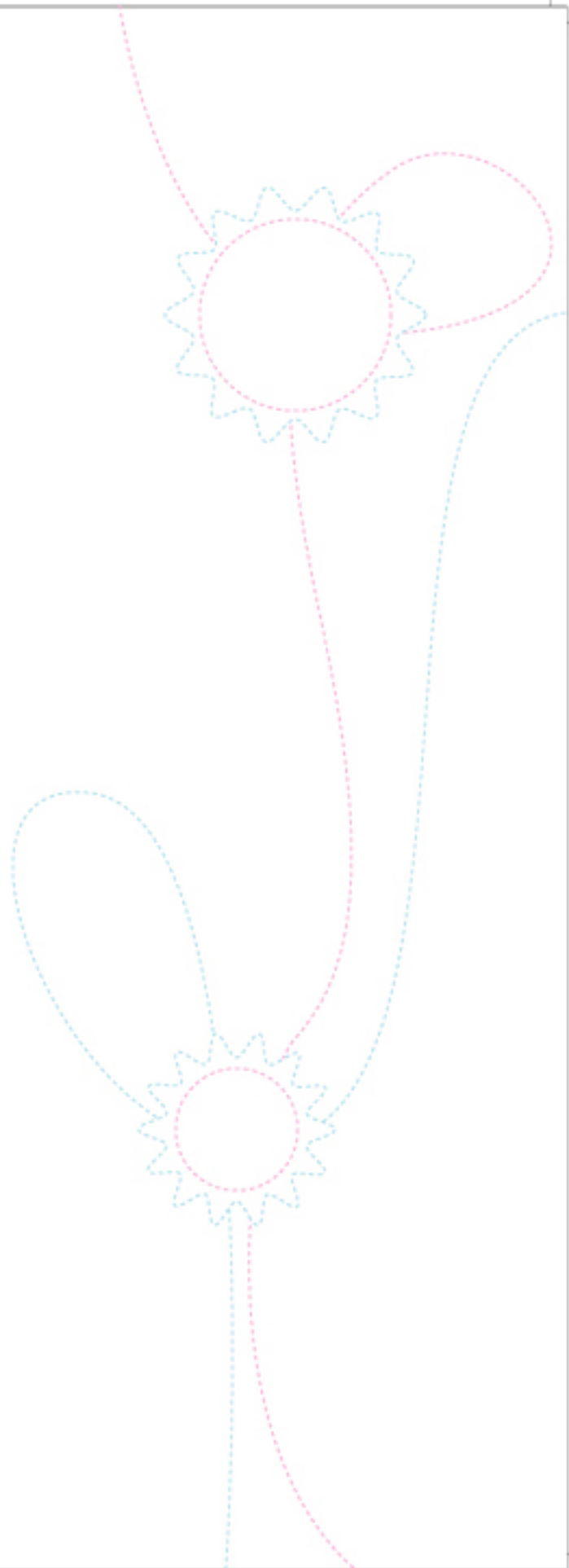
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**ISBN Number:**

000 DUMMY 000

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## Acknowledgements

The HIV program of CARE Bangladesh provides peer educator training to volunteer drug users who agree to take part in the training on HIV and STI prevention. After the training the participants agree to provide the information to their family, friends and other drug users informally during the course of their daily activities. The program participants, as peer educators, thus assume an active role in spreading the message on HIV prevention.

The HIV program would like to acknowledge the contributions of the peer educators in the development of this manual. Their feedback was essential in finalizing this manual.

The first draft of this manual was developed in 2003. Over the years, it has undergone several revisions and improvements. The following staff members of CARE Bangladesh have made valuable contributions at various stages in the development of this manual:

Dr. Somorjit Jana, Ronan Gomez, Md. Selim Khan, Md. Abul Kashem, Dr. Munir Ahmed, Dr. Md. Taslim Uddin, Md. Sakhawat Alam, Md. Mostafa Kamal, Harun-Ur-Rashid, Sonia Afrin and Mushfaqus Salehin.

In 2006-2007, HIV program began a process of reviewing various draft documents with an aim to bring them to publication. Peer Educator Manual is one of the documents reviewed and finalized by the support of Manzur Elahi from the Reflection and Learning Team. The original document was in Bangla language and its initial translation from Bangla to English was done by Lubain Masum.

The final revision and editing of the manual was done by Rukhsana Ayyub. The manual is being published both in English and Bangla for wider dissemination and sharing the learnings of the HIV program.

**CARE Bangladesh**



## Acronyms

|              |                                     |
|--------------|-------------------------------------|
| <b>AIDS</b>  | Acquired immunodeficiency syndrome  |
| <b>ANC</b>   | Antenatal care                      |
| <b>ARV</b>   | Anti-retroviral                     |
| <b>BCC</b>   | Behavior change communication       |
| <b>DIC</b>   | Drop-in center                      |
| <b>DU</b>    | Drug user                           |
| <b>HIV</b>   | Human immunodeficiency virus        |
| <b>IDU</b>   | Injecting drug user                 |
| <b>IEC</b>   | Information education communication |
| <b>IGA</b>   | Income generating activities        |
| <b>IV</b>    | Intra venous                        |
| <b>MSM</b>   | Men who have sex with men           |
| <b>NGO</b>   | Non-governmental organization       |
| <b>OHP</b>   | Over head projector                 |
| <b>OI</b>    | Opportunistic infections            |
| <b>OD</b>    | Over dose                           |
| <b>OW</b>    | Out reach worker                    |
| <b>PE</b>    | Peer educator                       |
| <b>PHA</b>   | People with HIV and AIDS            |
| <b>PLWHA</b> | People living with HIV and AIDS     |
| <b>SHG</b>   | Self-help group                     |
| <b>STD</b>   | Sexually transmitted diseases       |
| <b>STI</b>   | Sexually transmitted illnesses      |
| <b>TS</b>    | Transparency sheet                  |



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## Introduction

Bangladesh is at a critical stage in its response to the HIV and AIDS epidemic. Despite growing epidemics in neighboring India and South East Asia<sup>1</sup>, the HIV prevalence in Bangladesh remains very low.

National Sero-Surveillance data shows less than .01 percent HIV prevalence among the general population. However, among injection drug users (IDU) the HIV prevalence has slowly climbed, starting from 1% in 1998 to 7% in 2006<sup>2</sup>, and in one location in Central A, it is as high as 10%. This signifies the start of an epidemic among IDUs.

CARE-Bangladesh began working with drug users in 1998 and by June 2006, CARE was running one of the biggest interventions for the drug users in four of the six divisions of the country namely, Dhaka, Chittagong, Rajshahi and Sylhet including the hot spot areas of Dhaka. The program follows a harm reduction approach that has been shown to be the most successful and cost effective approach in reducing HIV transmission among IDUs<sup>3</sup>. Harm reduction is an umbrella of services concerned with minimizing the harmful effects of drug use and keeping the drug users safe and alive until they are ready and able to give-up drugs. The services provided by the HIV program include STI management, promoting safe injecting and safe sex practices, needle- syringe exchange, peer education, counseling and referral to drug treatment and detoxification programs. The program uses peer approach in which drug users are trained as peer educators since they are able to build the strongest rapport with the other drug users. This makes it easy to promote and disseminate prevention messages.

The program also engages outreach workers to implement the harm reduction program for IDUs. Most of them are recruited from ex and current drug users. The outreach workers are well trained on peer education and other components of harm reduction.

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1: 2006 report on the Global AIDS Epidemic UNAIDS

2: Bangladesh National Surveillance 2006

3: Burrows D (2003). HIV prevention among injecting drug users in transitional and developing countries. (draft). UNAIDS Best Practice Report



HIV Program has developed peer educators (PE) & outreach workers (OW) training module and a drug-counseling manual<sup>4</sup>. In addition the program has developed a core group of trainers for PE, OW and drug counseling training. The "Peer educator training manual for the prevention of STI and HIV and AIDS among drug users" is an informative document. It has been developed and standardized by incorporating up to date information and participatory training strategies on prevention of STI and HIV and AIDS. The manuals are being widely used in Bangladesh by CARE, its partner NGOs and many other local NGOs. Through the publication of these manuals, HIV program of CARE Bangladesh shares its resources and learnings with the development field and contributes to the national efforts to prevent HIV.

#### **Participants:**

The Peer Educators Training is given to drug users who volunteer to take part in it. It can also be provided as a refresher training to the peer educators. Outreach staff of IDU program can also participate in this training.

#### **Duration of Training:**

Program experience shows that it is not always possible to provide training to the drug users for extended hours. Trainers must be responsive to the state of the trainees present and adjust the length of the training in response to their needs. Timings given for each session are recommendations only. Training facilitators must constantly assess the participants in the training sessions. Session length can be adjusted depending upon the understanding and prior knowledge of the participants.

#### **Training venue:**

Field-level offices of HIV program, drop-in centers, partner organizations where 12-15 participants can receive training. Preferably sitting in 'U' shape to promote communication.

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<sup>4</sup>: Understanding drug addiction and counseling skills for the prevention of HIV, CARE Bangladesh 20071: 112: Bangladesh National Surveillance 2006

**Objectives of training:**

After the training, the participants will know and be able to explain:

- > HIV and AIDS. Mode of spread and ways of prevention
- > Harms associated with drug use
- > Importance of safe injecting practices
- > Risk of HIV infection from drug use
- > Signs and symptoms of STIs
- > Mode of spread of STIs and ways of prevention
- > Safe sex and condom use
- > Correct use of condoms
- > Skills, roles and tasks as peer educators
- > Role of peer educators in HIV prevention
- > Empowerment and building solidarity through joining self help groups.

Through this training the participants will be motivated to adopt healthy and safe behaviors and assume leadership role as peer educators to spread the message of healthy behaviors in their communities.

**Training methods:**

Presentation, demonstration, video show, practical exercise, group discussion, question and answer, brainstorming, role-play, card games and open discussion. The facilitator should use training methods that are of interest to the particular participants in each training session. Alternative training methods may be used considering the participants' attitudes, interest and previous learning.

**Training materials:**

Flip chart, flash card, flip paper, over head projector, video film, television, penis model, condoms, transparency sheet (picture), game items, display materials, board, marker clip etc.

**Information update:**

The manual contains information and data updated as of the printing date.



> Notes

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# Peer Educator Training Schedule

| Time       | Topic                                                                                                                                                           | Methods                                                                    | Materials                                                                                     |
|------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------|
| 0900- 0935 | Welcome.<br>Introduction of participants.<br>Training guidelines.<br>Expectations of the participants.<br>Objectives of training.                               | Speech,<br>introduction,<br>games, Q&A                                     | Poster paper,<br>marker, masking<br>tape, OHP, TS,<br>name cards,<br>lottery paper,<br>basket |
| 0935- 1020 | Basic facts about HIV/AIDS.<br>How HIV spreads and how it does not<br>spread.<br>Mode of prevention.<br>Global and Bangladesh perspective.                      | Presentation, open<br>discussion, games,<br>Q&A, OHP slide<br>presentation | Flash cards, slide,<br>OHP, marker pen,<br>flip paper, white<br>board                         |
| 1020-1050  | Understanding drugs and drug addiction.<br>Psychological and physical problems<br>from drug use.<br>Family and social problems from<br>drug use.                | Q&A, discussion,<br>presentation                                           | Flip paper, markers,<br>clip, tape, white<br>board, OHP, TS,<br>flash cards                   |
| 1050-1105  | <b>Tea break</b>                                                                                                                                                |                                                                            |                                                                                               |
| 1105- 1135 | Drug sharing<br>Overdose<br>Safe Injecting                                                                                                                      | Q&A, discussion,<br>presentation,<br>OHP presentation                      | Flip paper, OHP,<br>slide, marker,<br>flash cards                                             |
| 1135 -1205 | Signs and symptoms of abscess.<br>Causes and complications of abscess.<br>Treatment and management of abscess.<br>Cautions about abscess.                       | Q& A, discussion,<br>brainstorming                                         | Poster paper, OHP,<br>slide, marker                                                           |
| 1205- 1240 | What is STI? Different kinds of STI.<br>Mode of spread, causes, symptoms and<br>outcomes of STI.<br>Prevention of STI.<br>Relationship between STI and HIV/AIDS | Presentation, Q&A,<br>discussion                                           | Flip paper, OHP,<br>marker, booklet on<br>STI, slide                                          |



## Peer Educator Training Schedule

| Time       | Topic                                                                                                                                                   | Methods                                                    | Materials                                                         |
|------------|---------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------|-------------------------------------------------------------------|
| 1240- 0110 | Safe Sex and the Use of Condoms.<br>What is safe sexual behavior.<br>Proper use of condom and demonstration.<br>Pros and cons of using condom.          | Open discussion,<br>Q&A, presentation,<br>practice session | Flip paper, OHP,<br>marker, condom<br>card, penis model,<br>slide |
| 0110-0200  | <b>Lunch and Prayer Break</b>                                                                                                                           |                                                            |                                                                   |
| 0200 -0225 | Peer Education: What Is Peer? What is<br>peer education?<br>Skill and qualities of peer educator.                                                       | OHP, presentation<br>open discussion                       | OHP, TS, flip chart,<br>markers                                   |
| 0225-0310  | Empowerment and building solidarity.<br>Social position and human rights.<br>Empowerment.<br>Basic concept of SHG.<br>SHG of current and ex drug users. | Discussion, power<br>point presentation,<br>Q&A            |                                                                   |
| 0310-0330  | Evaluation and Conclusion.                                                                                                                              | Education, game,<br>feedback                               | Board marker,<br>question sheet                                   |





## **SESSION 1: STARTING THE TRAINING**

- > Welcome and introduction
- > Setting group guidelines for training
- > Clarifying expectations and objectives of the training
- > Annexure for session one

> Notes

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## > Welcome and introduction

### **Objectives:**

Participants will:

- > Be introduced to each other
- > Be motivated to participate in the training
- > Set guidelines for the training
- > Understand the objectives of the training

### **Required time:**

35 minutes

### **Methods:**

Games, question and answer, and discussion.

### **Materials:**

Appendix -1, board, marker, registration sheet, masking tape, basket, name cards, over head projector (OHP).

### **Process of facilitating the session:**

|        |                                                      |                 |
|--------|------------------------------------------------------|-----------------|
| Step 1 | Welcome participants and inaugurate training session | Time: 5 minutes |
|--------|------------------------------------------------------|-----------------|

Try to set a positive atmosphere from the start of the session. Request a colleague or a senior official to say a few words to welcome and motivate the participants.

|        |                               |                  |
|--------|-------------------------------|------------------|
| Step 2 | Ice breaker and introductions | Time: 10 minutes |
|--------|-------------------------------|------------------|

Invite all participants to take part in a game of introductions. Divide the participants in pairs (if uneven include yourself to form a pair). The participants share information about their name, profession, residence, likes, dislikes and hobbies to their pair in close discussion. Give five minutes, after which each pair is invited to introduce their partner based on the information they had obtained in the discussion. After all the pairs have been introduced the facilitator should introduce him/herself. Thank everyone for their participation and conclude the introductory session expressing hopes that pleasant environment will be maintained during the training session. Other methods or games may be substituted depending on the group make up.



**Step 3****Setting guidelines for the training****Time: 5 minutes**

We follow certain rules in our daily lives. Ask the participants if we should have some ground rules for the duration of the training as well. Brainstorm with the participants to identify some guidelines and write these on a flip chart. Ensure that all the important issues like listening to each other, respecting different views, being non-judgmental and confidentiality are covered. When everyone agrees with the guidelines, stick the flip chart in a prominent place for the rest of the training. Read out the guidelines written on the flip chart or ask some one to volunteer to read the guidelines. Encourage commitment to these guidelines.

**Step 4****Expectations from the training****Time: 10 minutes**

Ask participants to share what they expect out of the training. The participants can be divided into 3 or 4 groups and given poster papers and markers to write down their expectations. Ask each group to identify 4 or 5 expectations from the training. For those requiring assistance in writing, be prepared to offer help. Review the expectations and display in the training room for the duration of the training. Thank all for participation.

**Step 5****Training objectives****Time: 5 minutes**

Describe the objectives of the training to the participants by showing annexure 1.1 on OHP. Refer and relate each of the objectives to the expectations identified in the previous step. Encourage participants to actively participate in the discussion. Ask questions to see if participants have understood. If necessary, discuss and explain again.



## Annexure 1.1

### > Training Objectives

After completion of training, the participants will be able to know themselves and be able to explain to others basic information on the following topics:

#### **HIV and AIDS**

- > Modes of transmission
- > Ways of prevention

#### **Drugs and drug addiction**

- > Harms associated with drug abuse
- > Sharing and safe injecting

#### **Abscess**

- > Signs and symptoms
- o Abscess management

#### **STIs**

- > Types of STIs
- > Signs and symptoms
- > Mode of spread and prevention

#### **Safe Sex and condom use**

- > Types of condoms
- > Correct condom use
- > Clarify misconceptions

#### **Role of peer educator**

- > Skills and qualities of peer educator

#### **Empowerment and building solidarity**

- > Human rights
- > Empowerment
- > Self help group (SHG)
- > 2 SHGs for drug users

Through this training the participants will be motivated to:

- > adopt healthy and safe behaviors and
- > assume leadership role as peer educators to spread the message of healthy behaviors in their communities





> Notes

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## **SESSION 2: HIV AND AIDS**

- > Basic facts about HIV/AIDS
  - > Mode of spread
  - > Ways of prevention
- > Global and Bangladesh perspective
  - > Annexure for session two

> Notes

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## > Basic facts about HIV/AIDS

### **Objectives:**

Participants will learn about and be able to explain:

- > HIV/AIDS, mode of spread
- > Their responsibilities to prevent spread of HIV
- > HIV/AIDS pandemic in global and Bangladesh context.

### **Required time:**

45 minutes

### **Methods:**

Presentation, open discussion, OHP slide presentation, game

### **Materials:**

OHP, TS, board, markers, flash cards

### **Preparation:**

The facilitator should read annexure 2 in preparation.

### **Process of facilitating the session:**

#### **Step 1**

#### **What is HIV and AIDS?**

**Time: 10 minutes**

Inform the participants that we will discuss HIV/AIDS. Ask them if they know anything about HIV and AIDS. Encourage participants to give opinions. Listen to them and write down their opinions on a board. Present and explain slides 2.1 & 2.2

#### **Step 2**

#### **How HIV/AIDS spreads and how it does not**

**Time: 15 minutes**

Divide all participants into two groups and give each group two sets of playing card (flash card number 14-18 depicting scenes on how HIV is transmitted and how it is not). Participants can spread the cards on floor and divide them in to two piles: pile one to include cards that show how HIV is spread and pile two to include cards that show how HIV cannot be spread. Ask one group to check on how the other group divided the cards. Ask each group to defend and explain their choices of cards in each pile.



Now project flash card numbers 1-18 sequentially to explain the transmission of HIV. Later project picture number 19-20 and ask questions to the participants about the content of the pictures to test their understanding. Show slides 2.3, 2.4 & 2.5 to explain what causes AIDS. Throughout the session motivate participants to ask questions and provide clarifications. Discuss and explain each medium of transmission sequentially. Making sure participants understand that blood test is a prerequisite to identify HIV infection. Test the participants' learning about the overall discussion through question-answer. Summarize the discussion and highlight the importance of accurate information to reduce stigma and discrimination. Thank all and end this step. Depending upon the availability of time a game of " Fireball " can be played to highlight the importance of testing for HIV. The game consists of blindfolding the participants and giving them pieces of scotch tape. One participant has specially marked tape, which signifies HIV positive status. The participants do not know who has the marked tape. The participants are asked to walk around. As they bump into each other they will mark the person they bump into with the tape in their hand. Participants will continue to walk around the room giving and receiving the tapes from each other. After a few minutes unfold the blinds and let participants see whom they have shared with. Most people are surprised to find out how many of them have the marked HIV tape. Make the analogy with real life, when not knowing ones HIV status, one can spread the virus unknowingly. Emphasize the importance of knowing ones status.

**Step 3**

**HIV prevention**

**Time: 10 minutes**

With reference to the subject of the preceding step, ask the participants to describe ways of preventing HIV. Encourage them to express their opinions. Note down the gist of opinions on flip paper or board (small group may be allowed to discuss). Show slide 2.6 and provide required explanation for each of the mediums of HIV transmission and its prevention. Inform participants that the following sessions will build on and provide more information on prevention. Emphasize on following the prevention techniques in personal life. Test the participants' learning about the overall discussion through question-answer. Summarize the discussion. End this step with thanks to all.

**Step 4**

**HIV/AIDS: World and Bangladesh context**

**Time: 10 minutes**

Ask the participants if they want to know about the current state of the world on HIV and AIDS. Create interest among them to learn. Present the information on Bangladesh by showing slide 2.9 on OHP.



Explain the position of India and Myanmar around Bangladesh, in the Map. Explain about the frequent movement of people between these countries and the resulting risk of flow of HIV from one country to another. Now show slide 2.7 and explain the current state of HIV in Bangladesh. Emphasize the importance of containing the epidemic for a country with limited resources like Bangladesh. Motivate participants to think of ways to contain the epidemic and help them identify their role in HIV prevention efforts in the country.

### Annexure 2.1 What is HIV?

H = Human  
I = Immunodeficiency  
V = Virus

HIV is a virus that destroys the immune system of the body.

### Annexure 2.2 What is AIDS?

A = Acquired  
I = Immune  
D = Deficiency  
S = Syndrome

AIDS is a chronic, serious condition caused by the human immunodeficiency virus (HIV). By destroying the cells of the immune system, HIV reduces the body's ability to effectively fight off viruses, bacteria, and fungi that cause diseases. This makes a person more susceptible to certain types of opportunistic infections. AIDS refers to this late stage of an HIV infection.



### Annexure 2.3

#### How does HIV spread?

HIV spreads through the following body fluids:

1. Blood
2. Semen
3. Vaginal fluids
4. Breast milk

### Annexure 2.4

#### How is HIV transmitted from one person to another?

When an HIV infected person's blood or the body fluids enter the body of another person, the healthy person also becomes infected. Transmission mainly occurs in four ways:

##### **1. Sexual intercourse:**

Unprotected sexual relations with a man or woman increase one's chances of becoming infected with HIV. Using condoms each time with sexual intercourse decreases the possibility of infection by 85 percent.<sup>5</sup> A higher number of sexual partners also increase one's risk.

##### **2. Blood transmission:**

Getting transfusions of blood, which is HIV positive. This can occur through blood transfusions using untested blood.

##### **3. Use of needles and syringes:**

HIV is spread through sharing of needles or syringes as blood, plasma or corpuscles may remain in the used needle or syringe. Injecting drugs through shared needles and syringes is one of the most common means of HIV spread among injecting drug users.

##### **4. Mother to child transmission:**

HIV can be passed on from mother to child during pregnancy, childbirth or breastfeeding, if the mother is HIV positive.



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5. United States Department of Health and Human Services, National Institutes of Health, June 2000, "Scientific Evidence on Condom Effectiveness for Sexually Transmitted Disease (STD) Prevention Workshop."



## Annexure 2.5

### HIV is NOT spread by...

1. Drinking water from the same glass used by an HIV positive person
2. Swimming in the same pool or pond with an HIV positive person
3. Being bitten by a mosquito after it has bitten an HIV positive person
4. Mixing socially or living with an HIV positive person
5. Sharing the same toilet with an HIV positive person
6. Shaking hands with an HIV positive person
7. Hugging or kissing an HIV positive person
8. Sharing food, napkins or plates/silverware with an HIV positive person
9. Coughing or sneezing by an HIV positive person

**HIV virus spreads only by un protected sexual intercourse, shared needles and syringes, blood transmission and mother to child transmission.**

## Annexure 2.6

### HIV and AIDS prevention

#### **Preventing transmission through sexual intercourse**

- > Practice safe sexual behavior
- > Use condoms
- > Diagnose and treat sexually transmitted infections (STI) promptly

#### **Preventing transmission through infected blood**

- > Examine donated or purchased blood
- > Avoid unnecessary transmission of blood
- > Use sterilized medical equipment

#### **Preventing transmission by injection drug use**

- > Use sterilized or new needle and syringes to inject drugs every time
- > Do not share needles with anyone



**HIV positive and pregnant women can lower the chances of passing HIV to their unborn child by**

- > Regular follow up with the doctor
- > Getting tested for HIV
- > Taking a drug like AZT during pregnancy, labor, and delivery<sup>6</sup>
- > Giving medicine to the baby for the first six weeks of life.

**Reducing risky environment for STI and HIV and AIDS**

- > Learn about HIV and AIDS
- > Provide assistance and services in order to reduce risk
- > Creating a supportive environment to discuss sex
- > Do not show discrimination or stigma against HIV positive people

**ABCD Method: A proven effective prevention program**

Many countries and organizations have adopted an **"ABCD method"** originally developed in Uganda.

A = ABSTINENCE: Do not have pre-marital or extra-marital sexual relationship

B = Be FAITHFUL to your partner: Do not cheat on your spouse.

C = Use CONDOMS during sexual intercourse.<sup>7</sup>

D = Do not use DRUGS.



6. This service is not available in Bangladesh as of 2007.

7. Wawer, M.J., Gray, R., Serwadda, D., Namukwaya, Z., Makumbi, F., Sewankambo, N., Li, X., Lutalo, T., Nalugoda, F. & T Quinn, T. (2005). 27LB: Declines in HIV Prevalence in Uganda: Not as Simple as ABC. 12th Conference on Retroviruses and Opportunistic Infections. Session 8 Oral Abstracts: Diagnosis and Treatment of HIV Infection in Developing Countries. February 22-25, 2005.

## Annexure 2.7

### HIV/AIDS Pandemic:

#### State of the World (2007)<sup>8</sup>

|                                          |                                    |
|------------------------------------------|------------------------------------|
| Number of living people with HIV in 2007 | 33.2 million [30.6 - 36.1 million] |
| People newly infected with HIV in 2007   | 2.5 million [1.8 - 4.1 million]    |
| AIDS deaths in 2007                      | 2.1 million [1.9 - 2.4 million]    |

#### HIV/AIDS Pandemic: Bangladesh (2007)<sup>9</sup>

|                    |     |
|--------------------|-----|
| Total HIV Infected | 333 |
| Total AIDS Cases   | 125 |
| Total death        | 14  |

#### Cumulative as of 1st December 2007

|                      |      |
|----------------------|------|
| Total reported cases | 1207 |
| Total AIDS Cases     | 365  |
| Total death          | 123  |

8. AIDS pandemic update December 2005, UNAIDS  
9. 2006 report on the Global AIDS Epidemic UNAIDS

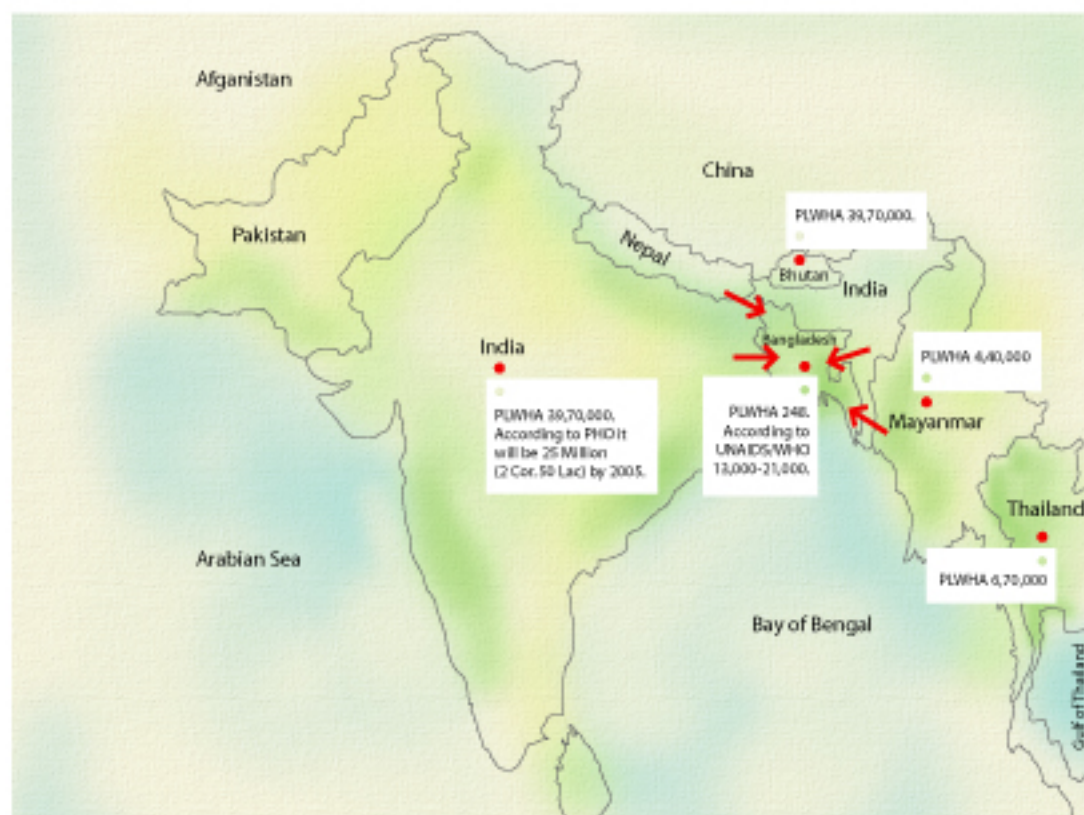


## Annexure: 2.8

### HIV and AIDS in Bangladesh

#### Risk of spreading HIV and AIDS in Bangladesh

Why is Bangladesh vulnerable? Although the number of HIV positive people in the country is small. Bangladesh is considered a high-risk country, due to its socio-economic conditions, geographical location, and the presence of certain high-risk behaviours.



#### Social reasons

- > High level of gender discrimination and inequality between men and women
- > Lack of awareness of HIV/AIDS among the general population
- > Marginalization, neglect and degradation of vulnerable populations
- > Lack of legal facilities
- > Lack of opportunities and initiatives of people to freely discuss sexual diseases and safe sexual practices. Religious and social norms do not encourage open discussion of sexual matters



**Economic reasons**

- > Migration (within and outside the country): People migrate from one place to another for employment and opportunity and live in risky conditions due to their profession, social status and situation
- > Financial stressors lead to drug use and risky behavior
- > Trafficking of women and children
- > Adoption of sex work as a profession due to poverty

**Behavioral reasons**

- > Low level of condom use
- > Inability to negotiate condom use due to gender and power imbalance
- > Youth become sexually active at an early age
- > High incidence of needle sharing among drug users

**Geographic reasons**

- > Four HIV affected countries surround Bangladesh: India, Myanmar, Nepal and Thailand.
- > Bangladesh citizens often travel to these countries as migrant workers, forced laborers or visitors.

## Annexure 2.9

**Populations most vulnerable to STIs, HIV and AIDS**

- > Sex workers
- > Injection drug users
- > Persons with multiple sex partners
- > Long-distance transport workers (driver and helpers), port and terminal workers, rickshaw-pullers
- > Women vulnerable to sexual abuse: garment workers, domestic help, victims of trafficking, sex workers
- > Men who have sex with men and Transgender



### **Drug addiction and HIV**

- > Injection drug users (IDU) have the highest rate of HIV prevalence in the country<sup>11</sup>
- > Many drug users sell their blood to collect money for drugs
- > Female drug users engage in sex work
- > Drug users migrate to other locations to obtain injection drugs
- > High incidence of sharing needles among drug users<sup>12</sup>
- > Surveys by CARE show low condom use by drug users when engaging in sex with sex workers

### **Reasons for vulnerability of women**

#### **Physical reasons:**

Biologically, the risk for transmission from male to female is greater than from female to male for several reasons

- > There is a greater exposed surface area in the female genital tract than in the male genital tract
- > There are higher concentrations of HIV in semen than in vaginal fluids
- > The vagina of young girls is not fully developed and prone to tearing
- > Coercive or forced sex might lead to micro lesions in the genital tract that facilitate entry of the virus.
- > Women often have STIs that are left untreated, which increases vulnerability to HIV.

#### **Socio-economic reasons**

- > Gender based violence
- > Illegal trafficking of women
- > Inability to negotiate condom use
- > Inadequate treatment facilities
- > Unavailability of condoms
- > Cultural preference for young girls as marriage partners
- > Economic dependence on men
- > Lower social ranking for women. Reports have also shown that 60 to 80 percent of HIV positive women in sub-Sahara Africa and 75% of women in Thailand were infected by their husbands, their sole partners<sup>13</sup>



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11. HIV prevalence increased from 1 %in 2000 to 7% in 2006 among the IDUs. ICDDR national surveillance report 2006

12. CARE Bangladesh, HIV/AIDS Program. Drug Users at Risk to HIV: Using Harm Reduction to Mitigate HIV. Documenting our Experience with Drug Users, 2000-2005.

13. Fact sheet: Women and AIDS: A Growing Challenge. UNAIDS 2004





### **SESSION 3:**

## **Drug addiction**

- > Defining drug addiction
- > Psychological and physical problems from drug abuse
  - > Family and social problems from drug abuse
  - > Annexure for session three

> Notes

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## > Defining drug addiction

### **Objectives:**

Participants will understand and be able to talk about;

- > Drugs, and drug addiction
- > Psychological problems created by drug use
- > Physical problems created by drug use
- > Family and social problems created by drug use

### **Required time:**

30 minutes

### **Methods:**

Question and answer, discussion, OHP slide presentation, flash cards

### **Materials:**

Flip paper, slides, board, markers, OHP

### **Process of facilitating the session:**

#### **Step 1**

#### **What is a drug?**

**Time: 10 minutes**

Raise the topic of drug addiction in this session. Project slide 3.1 with OHP. Discuss the types of drugs and commonly abused drugs in Bangladesh by showing slide 3.2 with OHP. Raise the topic of drug addiction. Invite all to comment on what they think addiction is and when a person can be considered to be addicted to drugs. Listen to their responses and write them down on poster paper. Now present slide 3.3 with OHP and explain addiction in a participatory approach. Focus on the drug user as a sick person in need of help and not a criminal.

#### **Step 2**

#### **Psychological and physical problems**

**Time: 10 minutes**

With reference to the earlier discussion, tell the participants that we will now discuss common problems caused by drug use. Invite the participants to share their thoughts on the psychological and physical problems they might have experienced due to drug use. Encourage all in discussion. Listen to their ideas and give one or two examples if necessary. Present slide 3.4 with OHP and start discussion in a participatory manner.



**Step 3****Social and family problems****Time: 10 minutes**

Start a discussion on the problems faced by the drug user in his family and social life. Expand the discussion to include problems faced by society at large from drug use. Help the participants take an honest inventory of the problems caused by their own drug use. Write down their responses on board or poster paper. Read out their responses at the end of discussion. Now present slide 3.5 with OHP and discuss the family and social problems caused by drug use. Do not blame the drug user, but keep the focus on the fact that the problems are caused by drug use and if the drug user was to stop drug use, the other problems could be slowly resolved. Thank all and end this step.

**Annexure 3.1****What is a drug?**

A drug is any substance, natural or artificially produced which when taken through mouth, inhalation or injection alters the normal functioning of the brain.

Generally speaking, drugs are medicines and materials that are used illegally without medical prescription.

Continued use of drugs leads to addiction

**Annexure 3.2****Types of drugs**

There are many drugs that people abuse. Most of the drugs are highly addictive. Drugs can be listed according to the following classification:

1. Narcotic analgesics
2. Stimulants
3. Depressants
4. Hallucinogens
5. Inhalants

**Commonly abused drugs**

- |                     |           |
|---------------------|-----------|
| > Nicotine          | > Alcohol |
| > Cannabis          | > Opium   |
| > Codeine phosphate | > Heroin  |
| > Buprenorphine     | > Cocaine |



### Annexure 3.3

## Drug addiction

#### What is Addiction?

Addiction is a progressive, chronic and potentially fatal condition. It affects almost every aspect of a person's life. It also affects the lives of those surrounding the person.

A person who is addicted to drugs shows:

> **Compulsion to use:**

strong thoughts, desires and physical craving to take the drugs. Thinking and planning about drug use all the time, also called addictive preoccupation.

> **Loss of Control:**

unable to control the time and or amount of drug taken. Frequent and failed attempts to control or stop drug use.

**Continued use in spite of problems in daily life** including health, family, relationships, employment, social and legal problems.

### Annexure 3.4

## Problems caused by drug use

#### Psychological

- > Reduced self care
- > Poor impulse control
- > Low frustration tolerance
- > Loneliness/ isolation
- > Guilt
- > Anxiety
- > Anger
- > Paranoia
- > Loss of motivation
- > Low self esteem
- > Depression
- > Suicidal intent



**Physical**

- > Poor health, Weakness/ Loss of appetite
- > Insomnia
- > Ulcers/Abscess
- > Liver disorder
- > Pulmonary complications including various types of Pneumonia
- > Stroke/ high blood pressure
- > Loss of immunity
- > STI/ Hepatitis/ HIV and AIDS
- > Jaundice
- > Physical and mental retardation in babies born to mothers who use drugs during pregnancy
- > Miscarriage and menstrual problems in women

**Annexure 3.5****Family and social problems caused by drug use**

- > Conflict with family members/ violence in the home
- > Loss of respect from family
- > Broken relations
- > Drop-out from school
- > Loss of job and income
- > Poverty in family
- > Loss of license as professionals
- > Criminal activity
- > Anti-social activity
- > Legal problems- arrest- imprisonment
- > Security breakdown in society





## **SESSION 4:** **Safe drug use**

- > Drug sharing
- > Overdose
- > Safe drug injecting
- > Annexure for session four

> Notes

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## > Safe drug use

### **Objectives:**

The participants will learn and be able to explain about:

- > Drug sharing practices
- > Overdose
- > Safe injecting

### **Required time:**

30 minutes

### **Methods:**

Question and answer, discussion, OHP slide presentation

### **Materials:**

Flash card (25th no.), flip paper, board, marker, OHP, and slide

### **Method of facilitating the session**

#### **Step 1**

#### **Drug sharing**

**Time: 10 minutes**

Ask the participants what they know about drug sharing. Encourage them to respond. Listen attentively to what they have to say. Present slide 4.1 on OHP to clarify and explain different types of drug sharing. Question the participants on the variety of ways of drug sharing. Link this discussion to the risk of spreading HIV and other illnesses through sharing. Encourage all the participants to take part in the discussion and to identify their own sharing patterns and risk behaviors. Promote safe behavior of one- shot one- syringe. Encourage only using own needle syringe and not giving ones own supplies to others. Clarify and add information if necessary.

#### **Step 2**

#### **Overdose**

**Time: 10 minutes**

Ask the participants about overdose. Encourage them to share about their experience of overdose. Provide the participants information on overdose by showing slide 4.2. Encourage them to ask questions to seek clarifications on some of the common causes of overdose. Come to a general consensus on the dangers of overdose particularly from cocktailing.



**Step 3****Safe drug injecting****Time: 15 minutes**

Divide the participants into 3 or 4 groups. Remind the participants that now they know how HIV is spread, ask them what they can do to ensure safe drug injecting. Make poster paper and pen available to all groups. Suggest selecting one person from every group to present the group work. Invite each group for presentation, when the stipulated time is over. After the presentations of all the groups present slide 4.3 on the OHP. Show picture number 25 to clarify which sites are safer to inject. Emphasize the golden rule of one- shot one syringe. Thank all and conclude the session.

**Annexure 4.1****Sharing**

To share drugs, needle, syringe or any other material used in the preparation of the drugs with one or more persons.

**Types of sharing**

- > Several people use one needle and syringe
- > To allow others to use one's own syringe, needle and other materials such as cotton, cooker or straw
- > To use syringe, needle and other materials such as cotton, cooker and straw belonging to someone else
- > To share ampoule with others
- > To wash or clean needle or syringe in single water pot being used by others
- > To use old needles and syringes

**Annexure 4.2****Overdose**

If a person takes too much of a drug, too fast, beyond the tolerance level of their body, dangerous side effects such as unconsciousness, stopped breathing, heart failure, or seizures may occur. This is known as a drug overdose. Overdose is very serious and can be fatal. Some common causes for overdose are:

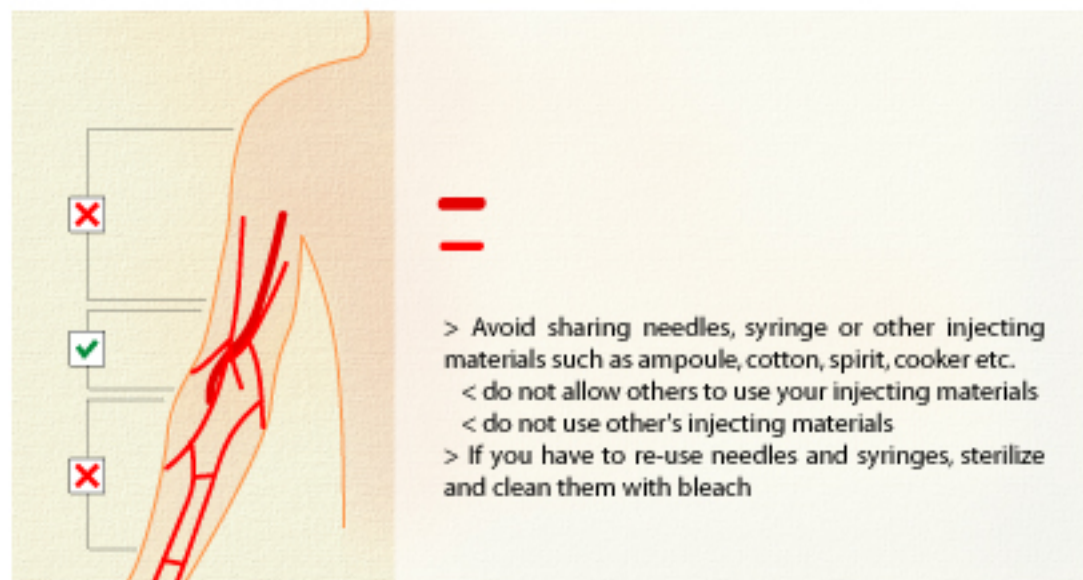
- > Using poor quality drugs
- > Using large quantity of drugs





- > Using drugs after a time gap. After detoxification or spending some time in a rehabilitation center, the body is no longer used to the same amount of drugs
- > Changes in health can increase risk for an overdose. Weight loss and liver problems can increase risk of over dose
- > **Cocktail:** To mix more than one drugs and take them together. For instance taking a mixture of Tidigesic, Evil, Sedil, Fenargan, and Easium. Cocktail increases the risk of overdose and abscess because drugs taken together can be much stronger than if they are taken individually.

### Annexure 4.3 Safe injecting



- > Avoid sharing needles, syringe or other injecting materials such as ampoule, cotton, spirit, cooker etc.
  - < do not allow others to use your injecting materials
  - < do not use other's injecting materials
- > If you have to re-use needles and syringes, sterilize and clean them with bleach
- > Clean the injection site before you inject
- > Use the smallest size needle
- > Change the injection sites frequently
- > Practice the golden rule: One shot one new needle and syringe



> Notes

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## **SESSION 5:**

### **Abscess management**

- > Signs and symptoms of abscess
- > Causes and complications of abscess formation
  - > Abscess management
  - > Annexure for session five

## > Notes

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## > Abscess management

### Objectives:

The participants will learn and be able to explain

- > Signs, symptoms and causes of abscess.
- > Basic information on abscess management

### Required time:

30 minutes

### Methods:

Question and answer, open discussion, brainstorming, presentation, film show

### Materials:

White board, board marker, flip paper, TS, OHP, and slide.

### Prior Preparation:

Facilitator to review the supplementary slides in advance

### Method of facilitating the session:

#### Step 1

#### Abscess and its signs and symptoms

Time: 10 minutes

Welcome the participants in the discussion on abscess. Ask the participants what they know about abscess. Encourage all to participate in the discussion. Write down their responses on flip chart. Participate in the discussion and provide accurate information. Show slide 5.1 "what is an abscess" and discuss with the participants. Next ask the participants how they will identify an abscess. Write down their responses on flip chart. Display slide 5.2 "the symptoms of abscess" ensure that the participants clearly understand the symptoms of abscess. Show slide 5.3 to add information on common sites for abscess formation

#### Step 2

#### Causes and complications of abscess formation

Time: 10 minutes

Draw the attention of the participants now to the causes of abscesses. Display slide 5.4 "**the causes of abscess**". Ask the participants whether they have any question. Invite participants to share personal examples if any, on abscess. Ask the participants to think about the consequences of abscess if treatment is not sought on time. Write down the feedbacks of the participants on flip chart. Show slide 5.5 "**complications of abscess**" to add to their knowledge and remove any misconceptions. In discussion, place emphasis on proper hygiene and cleanliness.



Referring to the discussion of the previous step, ask the participants to describe what they should do if they have an abscess. Encourage them to express their ideas. Write down the responses of the participants on flip chart. Display slide 5.6 "abscess management" and discuss the points by connecting them with the responses of the participants. Question the participants to know whether all of them have understood the issue clearly. Highlight the importance of proper and timely treatment. Ask the participants about any traditional treatment methods they know. Most common responses given are usually, use of lime, paste of pulse and poultice and application of heat. Inform the participants that these traditional cures can in fact be more harmful on the abscess. Highlight some precautions to be taken for the care of an abscess by displaying slide 5.6. Close the session by motivating all for self care and following proper abscess management. Offer thanks to all.

### Annexure 5.1

#### What is an abscess?

Abscess is an infection. It begins with redness, swelling, and tenderness at the injection site and develops into an infection with a hard, pus-filled core.

### Annexure 5.2

#### Signs & symptoms of abscess



- > A hard, warm lump starts to develop at the injection site
- > The affected part twinges with pain
- > The person feels feverish
- > Pain in groin or armpits
- > The abscess gradually becomes red and hard
- > Puss forms in it and it gets soft
- > If puss is formed, abscess might drain by itself or it can be opened and drained at a hospital or clinic

### Annexure 5.3

#### Common sites where an abscess is formed

Abscess can surface on any part of the body. For injecting drug users abscesses usually surface on the injecting sites. Abscess can be seen on:

- > Wrist
- > Arm
- > Armpits
- > Groin
- > Wounded area
- > Legs

### Annexure 5.4

#### Causes of abscess

Abscesses result from:

- > Reusing a needle or using a blunt needle
- > Sharing the needle and syringe
- > Missed hits (injecting into the tissue surrounding the vein)
- > Injecting a solution with a lot of particles in it
- > Dirty injection site
- > Using dirty injection equipment/ Improper sterilization of an old syringe
- > Pushing cocktail drugs in the flesh
- > Keeping the syringe uncapped/uncovered after using
- > Not removing all the air out of the syringe before filling with the drug.
- > Using drugs after the expiration date
- > Injecting in already infected sites

### Annexure 5.5

#### Complications of abscess

The following complications may be experienced if abscess is not treated in a timely and proper manner:

- > The abscess may become septic
- > The blood may be contaminated (septicemia)
- > Abscess may cause hole (Sinus)
- > Thin sinus/ulcer (fistula) may be formed
- > Inflammation of cells may occur (Cellulites)



- > Deep seated abscess may be formed
- > Amputation of a limb for removal of serious abscess

### Annexure 5.6

#### **Abscess management**

- > Clean the area with soap and water, keeping it as clean as possible at all times
- > Keep the wound site covered with sterile gauze
- > Consult the doctor if pus has accumulated in the abscess and needs to be drained
- > If the abscess is draining, let it continue to do so
- > Get the dressing changed on a regular basis. Properly dressing an abscess will help keep it free from further infection and speed healing
- > Dressings that directly touch the wound should be dampened with sterile saline and then covered with dry gauze and tape
- > Do not soak or use a compress once the wound is open or draining
- > After the abscess has drained and scabbed over, antibiotic creams can be helpful
- > Consult a doctor if there is bleeding
- > Take the full course of prescribed medicine.

### Annexure 5.7

#### **Some cautions**

- > Consult doctor if an abscess is unusually big. Do not dress or cut it on your own
- > Consult doctor if repeated treatment or taking antibiotic does not cure the abscess
- > Moreover never cut or squeeze any abscess yourself
- > Consult a doctor if drug taking points aches at the time of injecting drug, swells or grows pus
- > Fever, chills, extreme fatigue, or pain associated with an abscess, indicates a blood infection therefore seek medical attention immediately.







## **SESSION 6:**

### **Sexually transmitted infections**

- > What is an STI
- > Mode of spread, causes, symptoms and outcomes of STI
  - > Prevention of STI
- > Relationship between STI and HIV
  - > Annexure for session six

> Notes

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## > Sexually transmitted infections

### **Objectives:**

The participants will learn and be able to explain:

- > Sexually transmitted infections (STI) and different types of STI
- > Mode of spread, symptoms and outcomes of STI
- > Relationship between STI and HIV/AIDS
- > Importance of prevention and treatment of STI

### **Required time:**

35 minutes

### **Methods:**

Brainstorming, open discussion, presentation,

### **Materials:**

Flip paper, white board, marker, TS, OHP slide show, STI related HIV Communication Instrument 1.

### **Prior Preparation:**

Facilitator to review the supplementary booklet "HIV Communication Instrument 1" in preparation for this session

### **Process of facilitating the session:**

#### **Step 1**

#### **What is an STI? Different types of STIs**

**Time: 5 minutes**

For many people it is not easy to talk about sexuality and sexually transmitted infections. It is important to create an atmosphere where the participants feel comfortable discussing this topic. Facilitator should maintain a professional attitude and not joke about the topic. Explain to the participants the reasons for bringing up the topic. Ask participants if they know what STI is and to name some common STIs. Show slides 6.1 and 6.2 on the OHP and provide information. Show pictures of different types of STIs to help participants identify STIs. Be alert to the reactions of the participants.



**Step 2****Mode of spread, symptoms and outcomes of STI****Time: 15 minutes**

Ask the participants if they know any symptoms of STIs. Encourage them to express their opinions. Describe the symptoms for the different types of STIs. Display and explain slide 6.3. Ask the participants to think about the consequences of STIs. Encourage participants to think what would happen if STIs are not treated, write their responses on flip chart Explain in detail the serious outcomes that can result from STIs. Highlight the importance of timely diagnosis and treatment. Encourage questions.

**Step 3****Prevention of STIs****Time: 5 minutes**

Building on the previous session ask the participants, now that they know how STI is spread to think of ways of preventing STIs. Briefly raise the topic of safe sex, inform the participants that we will discuss this in greater detail in the next session. Make the link between drug use and STIs: Impaired judgment from drug use leading to high-risk behavior such as paid and unprotected sex. Finish the lesson thanking all for active participation.

**Step 4****Relationship between HIV/AIDS and STI****Time: 10 minutes**

Now ask the participants about the relationship between STIs and HIV. Encourage them in giving responses. Present slide-6.4 and discuss as required Show Figure: HIV/AIDS & STI to make the link between HIV and STIs: STIs increase the risk to HIV. Encourage participants to question until they are clear in their understanding. Thank all for participating.

**Annexure 6.1****What is Sexually Transmitted Infections (STI)?**

Sexually-transmitted infections (STIs), also known as sexually-transmitted diseases (STDs), are diseases that are commonly transmitted between partners through some form of sexual activity, most commonly vaginal intercourse, oral sex, or anal sex. Bacteria or virus cause STIs.

Bacteria cause syphilis, gonorrhea, chancroid and chlamydia while virus causes Human Immunodeficiency Virus (HIV), Herpes, Hepatitis B and Human Papillomavirus (HPV). A fungus, Candida, causes sexual transmitted infections.

| Symptoms of the infection   | Who it affects |
|-----------------------------|----------------|
| Urethral discharge          | Men and women  |
| Genital ulcer               | Men and women  |
| Vaginal discharge           | Women          |
| Pelvic inflammatory disease | Women          |
| Scrotal swelling            | Men            |
| Inguinal bubo               | Men and women  |
| Neonatal conjunctivitis     | Infants        |

## Annexure 6.2

### Different Types of STIs

- > Syphilis
- > Gonorrhea
- > Chancroid
- > Chlamydia
- > Trichomoniasis
- > Herpes Simplex
- > Warts of the anus or the sexual organs



### Annexure 6.3

## Causes, Symptoms and Outcomes of STIs

| Infection                                       | Cause                  | Symptoms                                                                                                           | Outcome                                                                                                              |
|-------------------------------------------------|------------------------|--------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------|
| Urethral infection generally known as Gonorrhea | Bacteria               | Discharge of pus from urethra, dysuria or burning sensation during micturation, increased frequency of micturation | Sterility, stricture of urethra, pelvic infection, neonatal conjunctivitis                                           |
| Genital ulcer generally known as Syphilis       | Bacteria               | Genital ulcer, blister or blistering ulcer with or without pain.                                                   | Risk of HIV infection, premature labour, Intra Uterine Death, Infection of nervous system and cardiovascular system. |
| Vaginal discharge                               | Bacteria and fungus    | Profuse foul smelling vaginal discharge, vulval itching, dysuria and dyspareunia                                   | Risk of HIV infection, pelvic infection, sterility, neonatal conjunctivitis                                          |
| Pelvic inflammatory disease                     | Bacteria and/or fungus | Pelvic pain, dyspareunia                                                                                           | Pain, sterility, pregnancy outside uterine cavity                                                                    |
| Scrotal swelling                                | Bacteria               | Scrotal swelling, severe pain in the scrotum                                                                       | Sterility                                                                                                            |
| Inguinal bubo                                   | Bacteria               | Bilateral or unilateral inguinal lymph node swelling                                                               | Genital swelling and fistula                                                                                         |
| Neonatal conjunctivitis                         | Bacteria               | Discharge of pus from eye, swelling or closing of eye lids                                                         | Blindness, ear infection, Otitis.                                                                                    |



#### Annexure 6.4

### Relationship between HIV/AIDS and STIs

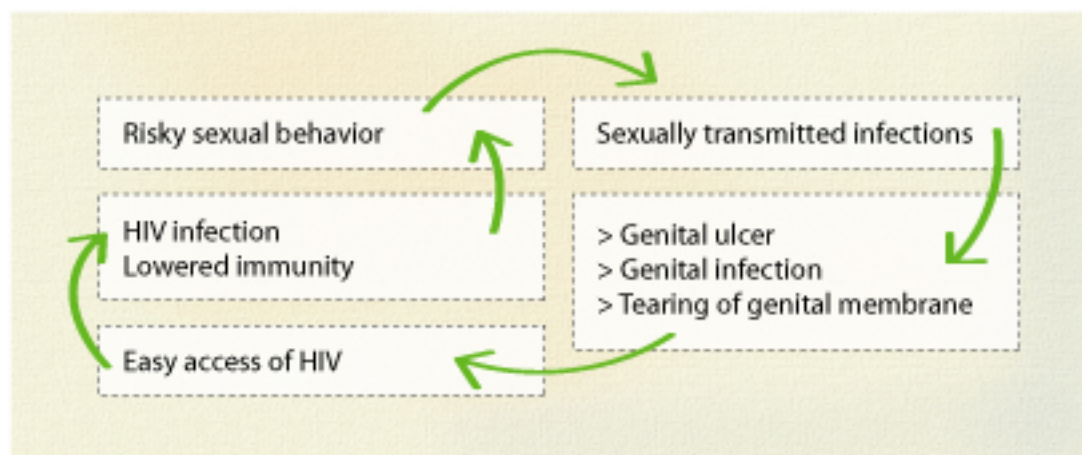


Figure: STI increases vulnerability to HIV

HIV/AIDS has close relationship with infection of reproductive organs. The diagram below explains this relationship.

- > HIV spreads the same way sexual infections spread
- > HIV like STIs spreads through unprotected sexual behavior
- > Presence of STIs increases the risk of HIV by creating open wounds and sores in the genital areas
- > HIV infection reduces the immune system which increases the risk of other infections including STIs.



> Notes

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## **SESSION 7:**

### **Safe sex and condom use**

- > Safe sex: different types of condoms
  - > Proper condom use
  - > Demonstrating condom use
- > Identify benefits and clarify misconception
  - > Annexure for session seven

> Notes

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## > Safe sex and condom use

### Objectives:

Participants will be able to:

- > Understand and describe the importance of safe sexual behaviors
- > Demonstrate proper use of male condoms
- > Learn and explain the benefits of using condoms and clarify misconceptions

### Required time:

30 minutes

### Methods:

Open discussion, demonstration, picture cards, game

### Materials:

Board marker, flip paper, penis model, condom, TS, OHP

### Process of facilitating the session:

#### Step 1

#### Safe sex

Time: 5 minutes

Ask the participants what they understand about safe sex. Motivate them to take part in the discussion. Question them about the importance of safe sexual behavior. Show slide 7.1 and 7.2. Explain the different types of condoms and their advantages in making sexual behavior safer by preventing the exchange of body fluids. Relate this to previous session on how HIV and STIs are spread. Explain the importance of checking the quality of condoms particularly the date of manufacturing and date of expiry on the condoms.

#### Step 2

#### Proper condom use

Time: 10 minutes

Divide the participants in two groups and give each group a set of condom cards. These cards depict the steps involved in correct use of both male and female condom. If the group consists of male participants use the cards for male condom only. For female participants use both male and female condom cards. Mix the cards up and ask each group to sort out the cards in the correct order of use. After the groups have sorted out the cards ask one member of each group to explain the picture cards to the larger group. Involve all in questioning and commenting. Present slides on OHP annexure 7.3 and walk the participants once again through the correct and proper procedure on using condoms from taking the condom out of the wrapper to the final disposal after use.



**Step 3****Demonstrating condom use****Time: 5 minutes**

Invite a volunteer to demonstrate the proper use of male condoms on a penis model. After the demonstration, invite responses from the participants. Use the information shown in the previous slide 7.3 on showing the correct mode of condom use. Distribute condoms and penis models to all the participants to practice correct use. Ensure participants are comfortable handling the penis model. Offer support to those who might express shyness. Highlight the importance of knowing correct condom use for self-protection.

**Step 4****Identify benefits and clarify misconceptions****Time: 10 minutes**

Invite the participants to discuss pros and cons of condom use. Divide the participants in small groups of 2 to 4 persons. Each group should elect its spokesperson to write and or report the views of the group. During presentations review the problems identified in condom use and try to clarify them as misconceptions by emphasizing the benefits of condom use. Show slides 7. 4 Reinforce consistent condom use for every sex act to prevent STI and HIV/AIDS. Complete the session by thanking all for their active participation.

**Annexure 7.1****Condoms**

Condoms are of two types:

**Condoms for men**

Condoms are sheaths of thin latex or plastic worn on the penis during intercourse. If used properly, latex condoms for men are highly effective at preventing STIs, including HIV and AIDS. They are also effective barrier methods of birth control.

**Condoms for women**

Female Condom is a soft, loose-fitting plastic pouch that lines the vagina. It has a soft ring at each end. The ring at the closed end is used to put the device inside the vagina and holds it in place. The other ring stays outside the vagina and partly covers the lip area. Female Condom is worn by women during sex to prevent pregnancy and STIs, including HIV infection.

Female condom is more expensive than the male condom and is not easily available in Bangladesh



## Annexure 7.2

### Practice safe sexual behavior

- > Build trusting sexual relationship
- > Abstain from unprotected sexual activities
- > Use condoms in all types of sexual acts: vaginal, anal, or oral sex.

#### **Use of condoms is an effective way of preventing HIV and STIs**

- > Avoid sexual activity if you have an STI
- > Practice non-penetrative sexual behavior; such as hugging, kissing, rubbing, licking, using thighs, arms and breasts of the partner for sexual gratification.
- > Get timely and proper treatment of STIs
- > If you have an STI, bring your partner for checkup and treatment

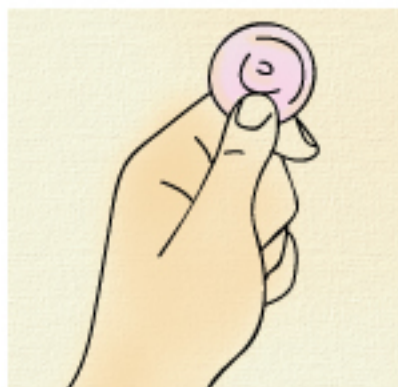
#### **Before using condoms:**

- > Ensure the quality of the condom
- > Examine the expiry date before using
- > Heat, light and air pollution can damage the condoms.
- > Keep condoms in a cool and dry place.
- > DO NOT use oil on LATEX condoms, as it will increase breakage
- > If needed use KY jelly for lubrication

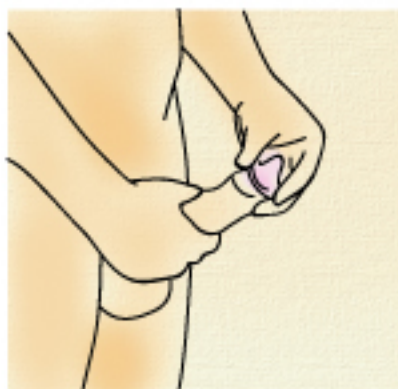


### Annexure 7.3

## Demonstration of correct condom use



1. Press the pack lightly to see whether the condom inside moves. Now tear off the pack carefully without using teeth or scissors.



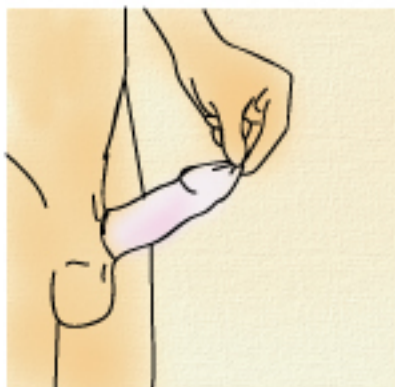
2. Check which way the condom unrolls. Hold the condom on the tip of the erect penis and unroll a little.



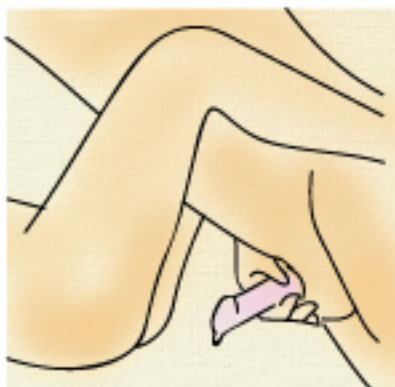
3. Now squeeze the air out of the tip and unroll the condom all the way down.



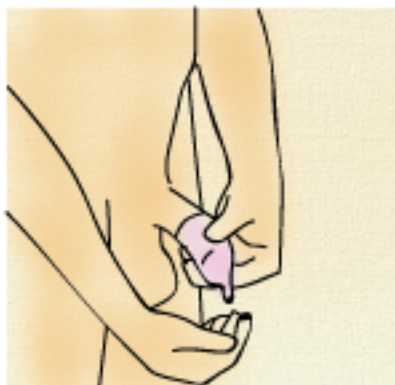




4. Ensure there is some space at the tip of the condom for collecting semen.



5. After ejaculation, hold the condom rim at the base of the penis and withdraw it while still erect.



6. Remove the condom gently. Do not let the semen spill from the condom. Now make a knot at the ring end of the condom, put it in the pack and dispose it in the dustbin.

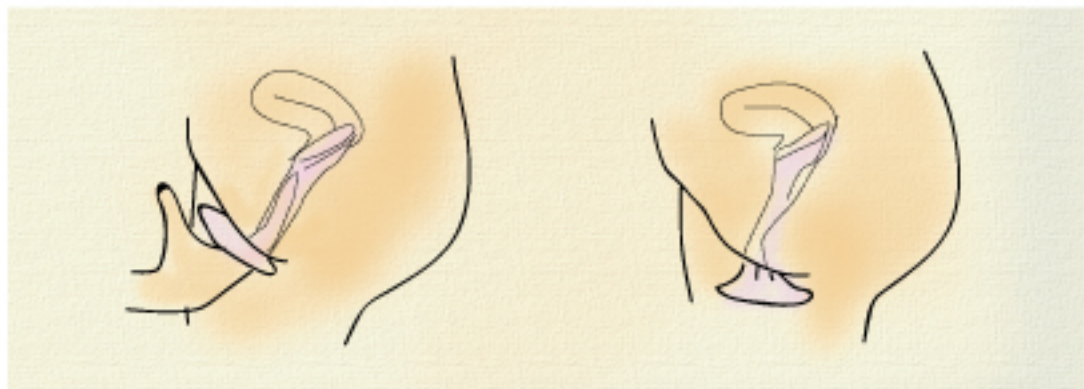


## Demonstrating correct use of female condoms



1. Open the pack and ensure by rubbing that the condom is well lubricated. If extra lubricant is needed oil or lotion may be used.

2. To insert the condom inside the vaginal track, choose a position with which you are easy; by sitting with legs drawn up; by squatting; by standing; or by lying while placing the weight on one leg.



3. The closed part of the condom will be inserted into the vagina. Using thumb and middle finger twist the ring of the condoms to form digit 8. Using two fingers push the ring into the vagina as deep as you can. Try to feel whether the inner ring has been inserted and placed properly or not. Proper attention should be given so that the condom and the ring do not twist or stand straight. The outer ring will remain outside the vaginal track.

4. Penis should be penetrated carefully so that it does not go out of the condom.





5. Remove the condom after intercourse. Now make a knot at the outer ring end of the condom, put it in the pack and dispose it in the dustbin.

6. After intercourse, remove the condom outside the vagina by wringing the outer ring of the condom very carefully.

#### Annexure 7.4

#### Advantages of using condoms for men

- > Condoms offer protection from STIs and HIV.
- > Condoms help prevent unwanted pregnancy.
- > Condoms create a sense of security.
- > Condom use indicates you are concerned and care about your partner.
- > Condom use will keep you healthy and save the cost of treatment
- > Condom use delays ejaculation and thus makes the pleasure last long.

#### Some misconceptions on condom use

- > Consistent condom use requires prior planning: As responsible adults we should only engage in sexual act when we are prepared, if we are not prepared then we can practice self-control or non-penetrative sexual behaviors
- > Condoms can tear or break: If you learn to store and use condoms correctly the chances of breakage are less
- > Some cost is involved in purchasing condoms: this cost is far less than the cost involved in treatment of STIs
- > It makes sexual intercourse less pleasant and abnormal. Actually by delaying ejaculation it can add to your pleasure
- > Some people maybe allergic to condoms: These days condoms are made with non allergenic material and it is very rare to find someone who is allergic to condoms
- > Some people hesitate to propose using condoms as this might indicate they do not trust their partner. In fact using condoms helps promote greater trusting relations by conveying to the partner that you care for their health and well-being.



### **Advantages of using female condom**

- > Female condom can be used for prevention of STI /HIV as well as a family planning tool for the prevention of pregnancy
- > Female condom is under the control of women as such it is a choice available to woman if men refuse to use male condom
- > The female condom is made of polyurethane, which diffuses heat; this creates a pleasurable sensation for the male partner during sex
- > The outer ring of the female condom rubs against the clitoris and produces a pleasurable sensation for women
- > Female condoms can be inserted in to the vagina up to 8 hours prior to sex.
- > Female condoms can be used during menstruation
- > Women of all ages can use the female condom. It is especially suited for women who experience side effects from hormonal contraceptives
- > The female condom is made of durable polyurethane plastic this reduces the possibility of tearing during sex
- > Women can use any type of lubrication with the female condom.

### **Some concerns on using female condom**

- > It requires prior preparation: With practice it can be inserted quite quickly. Advance preparation is an added benefit
- > If it is not set properly, it can get loose: practice and use can remove this fear
- > It involves some cost: this cost is less than the cost of treatment for STI or HIV
- > Sexual intercourse may seem less pleasant to those who are not familiar with it: Use over time can remove this misconception
- > Many women hesitate to propose the use of condom to close partner: Power imbalance between men and women in most cultures makes it difficult for women to insist on condom use. This is a larger issue the society has to tackle however, many women find unique and innovative ways in individual relations to convince their partners to use condoms for promoting health of both partners
- > Lack of understanding on correct usage of condoms: Practice makes perfect
- > If the rings are not set properly, the male partner can feel pain: Through practice and correct insertion this problem can be overcome.





## **SESSION 8:**

### **Peer education**

- > Peer and peer educator
- > Advantages of peer educator approach
  - > Skills and qualities of peer educators
  - > Annexure for session eight

> Notes

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## > Peer education

### **Objectives:**

Participants will be motivated to assume the role of Peer Educator. In addition participants will learn and be able to explain:

- > Peer and Peer Educator.
- > Roles of Peer Educators
- > Required skills and qualities of Peer Educator

### **Required time:**

25 Minutes

### **Method:**

Open discussion, presentation, small group discussion, demonstration

### **Materials:**

OHP, poster, slides

### **Process of facilitating the session:**

#### **Step 1**

#### **Peer/Peer Educator**

**Time: 5 minutes**

Start with an open discussion to clarify the meaning of Peer and Peer Educator for the participants. Encourage the participants to ask questions and give opinions. Make sure through question and answer that they have understood the concept. Present slide-8.1 and explain the peer approach.

#### **Step 2**

#### **Advantages of the Peer Educator approach**

**Time: 10 minutes**

Ask the participants if they think it is important to prevent STI/HIV/AIDS, if so what roles can they play for the prevention of HIV, after completing this training. Invite all to think and share their ideas. Encourage them to take part in the discussion and note down their responses on poster paper. Ask them about the advantages of the peer approach over traditional modes of giving training by experts. Motivate participants to think about the important role they can play as peer educators. Present slide-8.2 and discuss as required.



**Step 3****Skills & qualities of Peer Educators****Time: 10 minutes**

Ask the participants about the skills and qualities necessary to be a good Peer Educator. Motivate all to discuss and give opinions. Write down their responses on poster paper. Present slide-8.3 with OHP and compare with their responses. Recap the overall lesson through question and answer. Thank all for participation in the group work.

**Annexure 8.1****What is Peer**

Peer is an English word, which means companions, fellow, close friend or a person belonging to the same group. One drug user is peer to another drug user; one sex worker is peer to another sex worker.

**Role of a Peer Educator**

Peer educators are members of the participant community being targeted who are trained on safe behavior with the expectation that as an educated community member they will disseminate the information on their own, thereby making safer behavior practices sustainable, based on a true peer education and peer pressure approach.

**Annexure 8.2****Advantages of Peer Educator Approach**

- > Peer Educator approach empowers the community members by making them active self driven forces promoting healthy behaviors rather than passive recipients of information from outside experts
- > It promotes solidarity among the community members
- > It is a sustainable approach as the information imparted to the community will continue beyond the project life.



### Annexure 8.3

#### **Required qualities of Peer Educators**

The selection criteria includes the following:

- > Ex or current drug user, i.e. belonging to the same profession, life style and or social community
- > Willingness and interest to provide information to others
- > Able to maintain confidentiality of his peers
- > Possesses knowledge of the community
- > Acceptance within the community
- > Is a role model to his peers and practice safe behaviors
- > Shows good communication skills
- > Demonstrates good understanding of HIV and AIDS
- > Performs well in the training
- > A good listener



> Notes

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## **SESSION 9: Empowerment and Building Solidarity**

- > Social position of DUs and human rights
  - > Empowerment
- > Basic concept of SHG and advantages of joining SHGs
  - > SHGs of current and ex drug use
  - > Annexure for session nine

## > Notes

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## > Empowerment and Building Solidarity

### Objectives:

Participants will be motivated to join the self-help group (SHG). Participants will learn and be able to explain:

- > The social position of drug users and human rights
- > Basic concept of empowerment
- > Basic concept of SHG and advantages of joining SHG
- > SHGs of Current and ex-drug users

### Required time:

45 minutes

### Methods:

Lecture, group discussion, question & answer, power point/TS presentation

### Materials:

Flip chart board, flip chart paper, board marker, TS, OHP, multi media.

### The process of facilitating the session:

#### Step 1

#### Social position of DUs and human rights

Time: 10 minutes

Welcome all participants and explain the objective of this session. Start with giving some example and encourage participants to share about the problems they face in their daily lives. Listen to all their responses and write them down on a flip chart. Add your comments along with their comments if necessary. Show Annexure 9.1 and discuss the basic human rights that each individual is entitled to and the denial of rights particularly for the marginalized groups.

#### Step 2

#### Empowerment

Time: 10 minutes

In continuation of the previous discussion, ask participants to think of what they could do to change and improve their present social position and status. Ask them what is empowerment. Encourage them to participate in the discussion and listen to their comments. Link their discussion with the concept of empowerment. Now show annexure 9.2 on OHP. Discuss the importance of empowerment to establish dignity, social justice and rights in the society. Close the session by thanking all for their participation.



**Step 3****Basic concept and advantages of joining SHG****Time: 15 minutes**

Ask the participants how empowerment can be achieved. Encourage them in the discussion. Join the discussion and convey the message that to achieve dignity and rights in the society, they have to be united. A single drug user alone cannot achieve his/her rights and dignity in the society, but together, they can work towards establishing social justice, rights and dignity in the society. Ask comments from the participants on this issue and encourage all to discuss. Raise the concept of SHG showing annexure 9.3. Describe the important activities of the SHG and their role in helping their members in problem solving and achieving different rights i.e. reduce harassment, job placement and attaining social justice. Arrange a game on unity and discuss the lesson learned from this game.

**Step 4****SHGs of Current and ex-drug users****Time: 10 minutes**

Make a presentation on PROCHESTA and BODAR's organizational structure, activities and achievements for its member's welfare. Show annexure 9.4 by using multimedia and discuss. Encourage participants to identify the benefits of joining these SHGs. Review the whole session using the question and answer method. Encourage participants to ask question for further clarification. Thank all for their active participation.

**Annexure-9.1****Social position of DUs and human rights****A. Social position of DUs**

Drug Users are:

- > Extremely poor
- > Illiterate
- > Home less, living on streets
- > Lack self esteem
- > Involved in many socially unacceptable activities like petty theft and crime
- > Lack solidarity with other drug users
- > Vulnerable for Hepatitis-B/C and HIV transmission
- > Detached from family
- > Lack access to health service and information
- > Stigmatized by the society



## **B. Rights of the drug users:**

- > Access to quality health care and other services
- > Rights to take decision without any discrimination and threat.
- > To participate equally in all social, cultural and political events
- > Equal access to education and information
- > Access to legal support
- > Not to be discriminated and violated
- > Access to get job/work
- > Equal safety and security
- > Freedom of speech and movement
- > Access to shelter
- > Confidentiality and privacy
- > To establish an organization or to be a member of any organization

## **Annexure 9.2:**

### **Basic concept of empowerment**

**Empowerment means being able to control one's own life. It gives:**

- > power to make decisions
- > power to have your voice heard
- > power to negotiate new issues
- > power within yourself to challenge past customs.

## **Annexure 9.3:**

### **Self Help Group (SHG)**

From our childhood we hear sayings like

- > 'Self help is the best help'
- > 'Unity is strength'
- > 'United we stand, divided we fall'

#### **Example:**

We have heard the story of the birds caught in a net. They could not escape one by one. But when they flew together in a group, they escaped.

#### **Example:**

We know that one stick is easy to break. But it is difficult to break a stack of sticks.



It shows us how self-help is the best help. The SHGs build on this concept, through promoting solidarity for achieving the self defined interest and needs of its members.

- > Through forming SHGs communities plan, implement, monitor, and evaluate their development activities according to their needs and, or rights
- > The process requires mobilizing the resources within the community and is based on the community's own strengths.

The advantage of being member of SHGs

- > Feel less isolated knowing others share similar problems
- > Exchange ideas and effective ways to cope with the problems
- > Actively work on their attitudes and behavior to make positive changes
- > Gain a new sense of control over their lives
- > A sense of ownership of the activities of the group
- > Work towards HIV Prevention and welfare of the community
- > Collective empowerment to address problems and discrimination

#### Annexure 9.4: Two SHGs of drug users



প্রচেষ্টা

1. **Prochesta**; a self-help group of current injecting drug users was established in February 2000. It has a membership of 70 current drug users.

##### **Objectives of Prochesta:**

- > To organize the drug users
- > To establish their dignity in the society
- > To establish their fundamental rights
- > To reduce harassment by police and local mastans
- > Improved self-care through participatory involvement in the HIV/AIDS prevention activities
- > Communication and liaison with other GOs/NGOs
- > Participate in the different forum with a view to know more about HIV/AIDS
- > Advocacy both at National and Local levels.



**Major activities of Prochesta:**

- > Participates and ensures health education for the prevention of HIV/AIDS and STD among the IDUs
- > Represents the IDUs at different national and international forums
- > Participates in the different research studies
- > Conducts cultural activities consisting of drama, folk songs and Gumbira to increase awareness
- > Conducts advocacy sessions at local level.
- > Participates in observing World AIDS day and World Drug Free day

**Achievements:**

- > Prochesta has been able to increase its membership to 476
- > Has an 11 member elected executive committee
- > Formed 11 local committees
- > Gave birth to another organization named "BODAR" for the ex-drug users
- > Helps its members who are arrested through group advocacy with the police
- > Refers people to detoxification camps. Being a member of Prochesta increases the likelihood of getting admission into detoxification camp.



2. **BODAR** (Bangladesh Organization of Drug Addict Rehabilitation) is a SHG of ex-drug users. It was established in February 2001 with 15 recovering drug users. Their intention was to:

- > improve self esteem and image in the family and society
- > create acceptance within their families
- > regain social empowerment

**Objectives of BODAR:**

- > To organize detoxification for IDUs and initiate rehabilitation.
- > To raise awareness among children, youth and adults on drug related harms
- > To established linkages with GO, NGO and other volunteer organizations
- > To reduce HIV/AIDS related harm
- > To participate in relief efforts during national disasters
- > To established rights and social justice for the drug users.





**Major activities of BODAR:**

- > Organize and manage detoxification camps
- > Provide individual, family and group counseling.
- > Ensure follow up and home visits for drug users completing detox
- > Participate in different forums
- > Advocacy for social acceptance/ employment and reintegration in the society
- > Arrange N. A. (Narcotics Anonymous) meetings
- > Observe different national and international Days such as World AIDS Day and the International Day against Drug Abuse and Illicit Trafficking

**Achievements:**

- > The current strength of BODAR consists of 67 members
- > BODAR selects an 11 member Executive Committee
- > Participates as data collector in different surveys
- > Participated in 5th Round National sero-surveillance program in collaboration with ICDDR,B and CARE.
- > BODAR members participate in different cultural programs.







## **SESSION 10:**

### **Training Evaluation and Conclusion**

- > Training evaluation
- > Concluding session
- > Annexure for session ten

> Notes

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## > Empowerment and Building Solidarity

### **Objectives:**

The Participants will be able to:

- > Evaluate the training
- > Provide suggestion that can help make improvement in the next training course
- > Use the learning gained through participation in the evaluation session in the future.

### **Required Time:**

20 Minutes

### **Methods:**

Open discussion, presentation.

### **Materials:**

Board, marker pen, strips of paper containing questions, basket

### **Process of facilitating the session:**

**Step 1**

**Training evaluation**

**Time: 15 minutes**

Arrange a game to evaluate the training session. Prior to the start of the game, write down at least twenty questions on thin strips of paper. Sample questions are given in annexure 9. Place the folded strips into a basket. Divide the participants into two groups. Ask one member of each group to come forward, pick a question from the basket and read it out loud to the opposing group. The group members can consult with each other and then give the answer. Both groups take turn asking a question to each other. Facilitator provides feedback on the responses and keeps a scoreboard. Ask a minimum of ten questions to each group. Keep the atmosphere light and lively. Encourage healthy competition, making sure those who do not know the correct answer are not ridiculed or made to feel bad. Praise all for taking part in the game. Some token prize can be given to the winning group and an encouragement prize to the second group.



**Step 2****Concluding Session****Time: 5 minutes**

For the last session some cultural event can be planned. Participants can be invited to perform or sing a song, making sure the performance convey a message that is consistent and relevant to the objectives of the trainings.

In the end, call upon a few of the participants to say something on the training sessions. Congratulate the participants for completing the training successfully. Say a few words to build motivation in the participants to carry the message on HIV prevention. Finish the session by thanking all for their participation in the training.



## Annexure 10

### Sample Questions for Training Evaluation

| #  | Question                                                                                                 | True | False |
|----|----------------------------------------------------------------------------------------------------------|------|-------|
| 1  | AIDS occurs due to the weakness in the immune system                                                     |      |       |
| 2  | Having sex without condoms can lead to STIs                                                              |      |       |
| 3  | Anyone - rich or poor, young or old, man or woman can have HIV                                           |      |       |
| 4  | You can find out if you are HIV positive through blood test                                              |      |       |
| 5  | HIV causes AIDS                                                                                          |      |       |
| 6  | HIV destroys the immune system of our body                                                               |      |       |
| 7  | An infected person may not know that he is HIV positive or is suffering from STI                         |      |       |
| 8  | If some one is strong and healthy, he will not get HIV/AIDS or STIs                                      |      |       |
| 9  | HIV can spread through hand shake                                                                        |      |       |
| 10 | Syphilis is an STI                                                                                       |      |       |
| 11 | A child in womb may get HIV from pregnant mother                                                         |      |       |
| 12 | You may get HIV from using public toilet                                                                 |      |       |
| 13 | HIV positive or AIDS patients bed sheet and utensils are harmless                                        |      |       |
| 14 | A person may get HIV by donating blood<br>If I give my needle syringe to some one else it is not sharing |      |       |
| 15 | Sharing of syringe and needle by injecting drug users can lead to HIV                                    |      |       |



| #  | Question                                                                       | True | False |
|----|--------------------------------------------------------------------------------|------|-------|
| 16 | Sharing of syringe and needle by injecting drug users can lead to HIV          |      |       |
| 17 | Only women are susceptible to HIV                                              |      |       |
| 18 | HIV is spread easily through unprotected anal sex                              |      |       |
| 19 | If you have more than one sex partner you are more vulnerable to HIV or STIs   |      |       |
| 20 | Proper use of condom in every sex act prevents STI/HIV                         |      |       |
| 21 | Should seek treatment immediately after abscess are formed                     |      |       |
| 22 | Each time a new syringe/needle should be used                                  |      |       |
| 23 | If you an STI you must inform your partner/partners                            |      |       |
| 24 | Untreated STI cause serious health problems                                    |      |       |
| 25 | HIV can be spread by kissing                                                   |      |       |
| 26 | If you have an STI you must get treatment right away                           |      |       |
| 27 | Abscess can be prevented by keeping skin clean and using new needles each time |      |       |
| 28 | One shot one syringe will keep me safe from HIV                                |      |       |
| 29 | Give two qualities of a good peer educator                                     |      |       |
| 30 | Peer educators can spread the message in most effective ways                   |      |       |



> Notes

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PEER EDUCATOR  
TRAINING MANUAL  
FOR PREVENTION OF HIV AND AIDS  
among heroin smokers and injecting drug users

